MAX HOSPITAL, BATHINDA

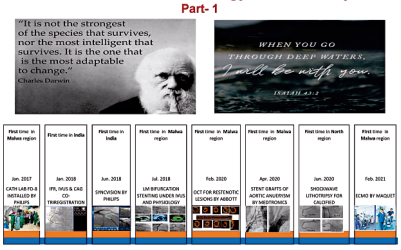
HEALTHCARE PAR EXCELLENCE



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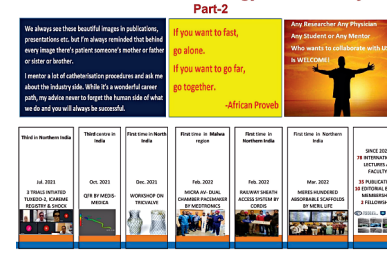
It is with a sense of immense humility that I'm describing our journey in interventional cardiology in Max hospital, Bathinda. We started our journey in 2017 when a new Cath lab was installed in this hospital. It was F10 model from Philips and such a modern Cath lab was installed for the first time by Philips in the Malwa region. We believe in the saying "*It is not the strongest species that survive, nor the most intelligent, but the one most responsive to change*". We have adapted to the rapidly occurring changes in cardiology and in 2018, we installed IFR, IVUS and CAG Co-triregistration by Philips. This type of facility is present in only a few selected centres in India, and it was the first one to be installed in North India. With this facility, we can know whether a patient requires a stent and the subjectivity of visualization by angiography alone is eliminated. With IVUS, we can deploy our stents and size them in a precise manner. It reduces long term complications. We also use sync-vision by Philips which also helps in performing angioplasties with accuracy. We were the first to do Left Main (LM) bifurcation stenting under IVUS and physiology guidance in the Malwa region in 2018.

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Since then, we have had a series of patients in whom LM stenting was done with good results. Without it, this subset of patients had to be treated with bypass surgery alone. In 2020, we were first in the Malwa region to use Optical Coherence Tomography (OCT) in patients for restenotic lesions. For the patients in whom stents get re-blocked, this imaging modality can improve the long term outcomes. In 2020, stent-grafts of aortic aneurysm were used for the first time in the Malwa region. An aortic aneurysm is a condition in which the aorta, a large blood vessel originating from the heart that distributes the blood to all organs of the body increases in size and is hence prone to rupture. Stent grafts are placed to prevent this. Extending our journey, in June 2020 we used shockwave lithotripsy for calcified coronaries for the first time. When the coronary arteries become calcified, putting a stent in them becomes difficult and dangerous. But with the help of shockwave lithotripsy, it can be made easy and optimal. This therapy was used for the first time in the whole of the northern region at our centre. By 2021, we started ECMO for very critically sick patients. We used ECMO in very critical patients in shock and have also saved a few patients from deadly poisoning. ECMO functions as temporary support to the heart and lungs of patients and can be helpful to tide over the crisis. "*When you go through deep water, I will be with you*". In the middle of 2021, we increased our research activities and initiated three clinical trials TUXEDO-2, iCaReMe registry and SHOCK India registry. The importance of clinical trials is that we can closely monitor the outcome of our patients and suggest new treatments. These research trials are generally done by cardiologists of foreign countries, and India is far behind in research activities. We, here in Bathinda, are among a few centres in India that are pursuing research activities in a very ethical and active manner. In late 2021, we installed a software for QFR. We are the third centre in India to do that. By QFR, we can quantitate the angiographic findings, and our decisions on whether to stent are more precise. We used absorbable scaffolds instead of stents first time in the Malwa region. The absorbable scaffolds vanish away after two years of deployment. This scenario can be beneficial for the human body in

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the long run. Another important product from CORDIS was the railway sheath access system. With this, we could reduce the pain experienced by the patient while doing angiography and angioplasty by the radial route. In February 2022, we successfully delivered MICRA-AV dual-chamber pacemaker by MEDTRONICS. It is a leadless pacemaker and is advantageous over the old pacemakers because it is devoid of the complications like pocket infection, lead displacement and fracture. This newer therapy which was used for the first time in northern India can be a boon for very sick patients who require a pacemaker. To summarize, we always see these beautiful images in publications, presentations etc. but I'm always reminded that behind every image there's a human who is someone's mother or father, sister or brother. I mentor a lot of catheterisation procedures and they ask me about the industry side. While it's a wonderful career path, my advice is to never forget the human side of what we do and I feel that is what makes one successful. Since 2020, we had delivered 78 international lectures as faculty of various international conferences and published 35 articles in renowned journals and earned 10 editorial board memberships and two fellowships. This is our journey at Max hospital Bathinda, in brief. We did many firsts not only in the Malwa region, but in the entire country. Our work gives its testimony. To add spices, we did satisfying academic work which has raised our name not only in India, but worldwide in countries like USA, Europe, China, South Korea and the UK, to name a few. I admit that this has only been possible because of good wishes from family and friends.

By Dr.Rohit Mody, Director of Cardiology, Max Superspeciality Hospital , Bhatinda.